

Scientific and Teaching Review: EMDR with Complex Trauma and Structural Dissociation (EMDR ARABIA, 4 days, draft)

Review date: 24 September 2026

1. Scope and method

Course files reviewed

- Trainer manual (دليل المدرب)
- Participant workbook (دليل المشارك)
- Slide decks for Days 1 (v2), 2, 3 and 4, including speaker notes
- Case record for Reem and Tarek (v2)
- Assistants' run sheet
- Course structure (v2)
- Course map PDF (its Arabic text did not extract, so it was not reviewed)

Sources checked against (supplied by the course author)

Source	What was read
Steele, Boon & van der Hart (2017), <i>Treating Trauma-Related Dissociation</i> (TTRD)	Full text
Boon, Steele & van der Hart (2011), <i>Coping with Trauma-Related Dissociation</i>	Full text
Knipe (2015), <i>EMDR Toolbox</i> , 1st ed.	Full text
Leeds (2016), <i>A Guide to the Standard EMDR Therapy Protocols</i> , 2nd ed.	Ch. 5 (pp. 119–136), pp. 160–163 (RDI), pp. 215–220 (interweaves), glossary pp. 447–450
Shapiro (2018), <i>EMDR Therapy</i> , 3rd ed.	pp. 220–222 (EMD)
Van der Hart, Nijenhuis & Steele (2006), <i>The Haunted Self</i>	Ch. 1

Other checks

- **Retractions:** I searched Crossref's Retraction Watch dataset (72,606 records, downloaded from Crossref's GitLab mirror) by DOI, author and keyword.
- **ICD-11:** the WHO browser (icd.who.int) is blocked from this environment. I checked ICD-11 wording against published reproductions of the WHO text instead.
- **Currency and later evidence:** checked by web search.

Limits

- Some items were accepted on the course author's confirmation without being checked here. They are marked **Confirmed by author** below.
- The pre-test/post-test (12 items) and its answer key were not supplied, so they could not be reviewed.
- Retraction Watch covers journal articles, not books.

2. Rating key

Rating	Meaning
Supported	The cited source says it
Overstated	Stated more strongly or more generally than the source allows
Misquoted	Differs from what the source says
Outdated	Matches the source, but later higher-quality evidence contradicts or qualifies it
Unsupported	Not found in the cited or supplied sources
Citation error	The claim may be true, but the reference is wrong
Confirmed by author	The course author confirmed it; not checked in this review

3. Summary

48 claims were rated:

Rating	Count
Supported	40
Confirmed by author	3
Outdated	2
Overstated	1
Misquoted	1
Citation error	1
Unsupported	0

- **No cited journal article has been retracted.**
- **One cited book has a newer edition:** Knipe, which is cited inconsistently (see section 5).
- **Teaching design is strong.** The main teaching weaknesses are four objectives that can't be measured, three objectives with no task that assesses them, and a test that wasn't available to review.

4. Claim-by-claim ratings

D = day, S = slide. Page numbers refer to the supplied excerpts.

Day 1: Understand and assess

#	Location	Claim	Rating	Evidence
1	D1 S23	ICD-11 PTSD has three clusters: re-experiencing in the present, avoidance, and a sense of current threat	Supported	ICD-11 6B40
2	D1 S24	CPTSD = all PTSD requirements plus problems in affect regulation, self-concept and relationships; usually follows prolonged or repeated events that are hard to escape	Supported	ICD-11 6B41: "most commonly prolonged or repetitive events from which escape is difficult or impossible"
3	D1 S26	Dissociation in the narrow sense = division of the personality (presented as the model's definition)	Supported	TTRD ch. 1: "the original meaning of the term"

#	Location	Claim	Rating	Evidence
4	D1 S27	Alterations of consciousness (absorption, narrowed attention) are common and are not dissociation in the narrow sense	Supported	TTRD: "We prefer not to call these phenomena dissociation"; cites Steele et al. (2009)
5	D1 S28	ICD-11 descriptions of DID (6B64), partial DID (6B65) and depersonalization-derealization disorder (6B66)	Supported	ICD-11 6B65: non-dominant states "do not recurrently take executive control... occasional, limited and transient episodes"
6	D1 S28	DSM-5-TR equivalents are DID, OSDD type 1 and DPDR	Supported (approximate)	Partial DID and OSDD-1 overlap but are not identical categories
7	D1 S29	PTSD, CPTSD, partial DID and DID lie on one spectrum, and stabilization lengthens along it	Overstated	ICD-11 classifies these as separate disorders. TTRD uses "continuum" only for DES items and countertransference. The spectrum is the structural dissociation model's view, not a diagnostic fact. Label it as the model's view
8	D1 S37	Action systems: daily life (exploration, attachment, caregiving, play, energy management) and defense (flight, fight, freeze, submission)	Supported	<i>Haunted Self</i> ch. 1; TTRD ch. 1. Omits hypervigilance, sociability and ranking (acceptable simplification)
9	D1 S38	ANP carries daily life and often avoids traumatic memories	Supported	<i>Haunted Self</i> ch. 1: ANP is "the dissociative part of the personality which avoids traumatic memory"
10	D1 S39	EP is fixed in defense and trauma and has its own sense of self	Supported	<i>Haunted Self</i> ch. 1: EP is "fixated in traumatic memories"
11	D1 S40	Parts are aspects of one person	Supported	<i>Haunted Self</i> ch. 1: "part of a single human being"
12	D1 S41	Primary level = one ANP + one EP, which corresponds to PTSD	Supported	<i>Haunted Self</i> ch. 1; Leeds pp. 129–130
13	D1 S41	Secondary level = CPTSD and partial DID; tertiary level = DID	Confirmed by author	Leeds p. 128 supports "OSDD secondary, DID tertiary". Placing CPTSD at the secondary level was not found in the supplied pages
14	D1 S67	Screen for dissociation with a tool before any bilateral stimulation	Supported	Leeds p. 161: "always screen for a dissociative disorder before offering any bilateral stimulation procedures"
15	D1 S68	DES-II has 28 items scored 0–100, and the score is their mean	Supported	Leeds p. 130
16	D1 S68	A DES-II score of 30 or more warrants further assessment	Supported	Carlson & Putnam (1993), as reported in secondary sources; the article itself was not read
17	D1 S68 notes	A low DES-II score does not rule out dissociation	Supported	Leeds p. 130: "a weak instrument that fails to identify the presence of dissociative disorders in a significant percentage of cases"
18	D1 S69	MID is a long multidimensional questionnaire; SCID-D and TADS-I are structured interviews	Supported	Leeds p. 135 (MID has 218 items; SCID-D-R). The TADS-I is semi-structured; minor
19	D1 S71	Previous attempts are among the strongest predictors	Outdated	Leeds p. 126 says it. Franklin et al. (2017) found every risk factor predicts only slightly better than chance, and NICE NG225 (2022) advises against risk prediction

Day 2: Safety, stabilization and readiness

#	Location	Claim	Rating	Evidence
20	D2 S6	Suicide and self-harm assessment is part of the readiness checklist	Supported	Leeds Exhibit 5.1 (bold items)
21	D2 S7	Ask about ideation, intent, plan, means, previous attempts, self-harm and protective factors	Supported	Leeds Exhibit 5.2: "intent, plan, and lethality"; "past suicide attempts need to be fully understood"
22	D2 S8	Ideation alone is a weaker indicator; previous attempts are among the strongest	Outdated	As #19. Leeds relies on Oquendo et al. (2004, 2006)
23	D2 S9	With dissociation, also ask about lost time, internal voices, and injuries or missing medication the client can't explain; there may be attempts the client doesn't remember	Supported	TTRD ch. 18: 60% of dissociative patients report amnesia around self-injury (Coons & Milstein, 1990); command voices; patients "unaware of highly suicidal parts"
24	D2 S10	When risk is high: a written safety plan, a psychiatrist, a clear treatment contract, memory work postponed, risk monitored every session	Supported	Leeds p. 126 (postpone reprocessing, work with psychiatrists, clarify the treatment contract and mandated actions); TTRD ch. 18 (written safety plan)
25	D2 S11	Current external threat versus a learned expectation	Supported	Leeds p. 124 (external crises: defer reprocessing)
26	D2 S24	The five phobias: inner experience, dissociative parts, traumatic memory, attachment and attachment loss, change and normal life	Supported	TTRD, almost verbatim
27	D2 S26	Avoidance persists because of the relief it brings	Supported	Knife: "avoidance carries a feeling of relief"
28	D2 S27	Addictive behaviour can be a form of avoidance defense	Supported	Knife ch. 1 lists "avoidance defenses, including addictive behavior patterns"; Knife ch. 6
29	D2 S38	Capacities to check include presence and dual attention	Supported	Leeds p. 129 (dual attention); Knife ch. 2
30	D2 S41	Stabilization components listed from <i>Coping</i>	Supported	<i>Coping</i> Parts 1-5
31	D2 S42	Phase 1 goals with a dissociative disorder: sleep, orientation to the present, gaps, simple inner communication, safety	Supported	<i>Coping</i> chs. 1-9; TTRD
32	D2 block 2.4	Explore the function of the part that wants to die while safety measures stay in place	Supported	TTRD ch. 18: "explore reasons why the patient, or a particular dissociative part, is suicidal"

Day 3: Formulation, preparation and target selection

#	Location	Claim	Rating	Evidence
33	D3 S32	Controlled studies on RDI are "few" (قليلة)	Misquoted	Leeds p. 160: "no controlled treatment outcome research"; p. 162: "no high-quality controlled research published on the full RDI protocol"
34	D3 S32	RDI needs informed consent and dissociation screening	Supported	Leeds pp. 161-162
35	D3 S33	A mastery resource should be close to the missing capacity	Supported	Leeds Exhibit 6.10, steps 1-2
36	D3 S34	Bilateral stimulation is an essential step in RDI	Supported	Leeds Exhibit 6.10, step 4 (sets of 6-12)
37	D3 S50	Three-pronged protocol: past, present, future	Supported	Shapiro p. 220

#	Location	Claim	Rating	Evidence
38	D3 S51	Default: the oldest or touchstone memory in the cluster first	Supported	Shapiro p. 220 target order; Leeds glossary p. 450 (touchstone "hypothesized to be etiological")
39	D3 S64	Hofmann's inverted protocol (future, then present, then past) for unstable complex cases	Supported	Leeds glossary p. 447
40	D3 S62-63	Indicators for starting memory work	Supported	A synthesis consistent with Leeds ch. 5 and TTRD

Day 4: Processing, integration and the final decision

#	Location	Claim	Rating	Evidence
41	D4 block 4.1	Leeds's question "What keeps the distress from going down?"	Confirmed by author	
42	D4 S44	Change direction or speed first, then use an interweave	Confirmed by author	
43	D4 block 4.3	A cognitive interweave is a small question, not an argument; use the least intervention first	Supported	Leeds p. 218: "offered in the form of a question... That's all that is needed before offering the next set"
44	D4 S48-49	CIPOS is for loss of presence near the memory, requires the ability to return to the present, and needs separate training	Supported	Knipe ch. 13; Back of the Head Scale
45	D4 S50	EMD narrows associations by returning to the target after each set	Supported	Shapiro pp. 220-221
46	D4 S51	Evidence for Flash is still limited	Supported	One RCT (Yaşar et al., 2022; traffic-accident survivors) and preliminary reviews. The ENHANCE RCT (PTSD) protocol was published in 2023; no results were found
47	D4 S52	Loving Eyes is an alternative to the Phase 3 questions when fear is too high or defenses block affect; it requires an adult perspective	Supported	Knipe ch. 11 and ch. 1 ("too much... or too little psychological defense")
48	D4 S53	IHD, cited as "Knipe ch. 14"	Citation error	Not in Knipe (2015); ch. 14 is a case chapter. The source is confirmed by the author, so correct the reference. Knipe ch. 11 does describe therapist-guided "internal dialogue" between parts
—	D4 S62-63	Realization; personification and presentification	Supported	TTRD ch. 1; Leeds glossary p. 449
—	D4 S64	Monitor risk at closure and at re-evaluation; stop memory work if risk rises	Supported	Leeds p. 126 ("ongoing monitoring"); TTRD ch. 18

The last two rows are general principles and are not counted in the 48.

The course author also confirmed four things that are rated on other grounds above:

- the DES-II cutoff (#16), rated Supported because secondary sources confirm it;
- the Flash article (#46);
- the source of IHD (#48), which is still a citation error because the reference given is wrong;
- the Knipe edition (see section 5).

5. Citation audit

Citation in the course	Exists	Peer-reviewed	Current?	Retracted?	Issue
WHO, ICD-11 6B40, 6B41, 6B64–6B66	Yes	Official classification	Yes	n/a	6B63 (possession trance disorder) is not covered; see Gaps
APA, DSM-5-TR (2022)	Yes	Official classification	Yes	n/a	—
Steele, Boon & van der Hart (2017), TTRD, Norton	Yes	Scholarly book	Yes (latest edition)	n/a	—
Van der Hart, Nijenhuis & Steele (2006), <i>The Haunted Self</i> , Norton	Yes	Scholarly book	Yes	n/a	—
Boon, Steele & van der Hart (2011), <i>Coping</i> , Norton	Yes	Scholarly book	Yes	n/a	—
Leeds (2016), 2nd ed., Springer	Yes	Scholarly book	Yes (no 3rd edition found)	n/a	Suicide-risk evidence dates from 2004–2012
Shapiro (2018), 3rd ed., Guilford	Yes	Scholarly book	Yes	n/a	—
Knipe, <i>EMDR Toolbox</i> " (2nd ed.)"	Yes; the 2nd edition was published in 2018	Scholarly book	Yes	n/a	The chapter numbers cited match the 2015 1st edition. Cite one edition consistently
Carlson & Putnam (1993), <i>Dissociation</i> , 6(1), 16–27	Yes	Yes	Still the standard DES-II reference	Not in the database	The journal <i>Dissociation</i> has no DOIs; checked by author and title
Dell (2006), <i>J Trauma & Dissociation</i> , 7(2), 77–106	Yes	Yes	Yes	Not in the database	—
Manfield, Lovett, Engel & Manfield (2017), <i>J EMDR Practice & Research</i> , 11(4), 195–205	Yes	Yes	Supersede with newer evidence (Yaşar et al., 2022)	Not in the database	A case series with four cases, not outcome evidence
Steinberg, SCID-D (no year given)	Yes	—	A newer DSM-5/ICD-11 version exists (APA Publishing)	n/a	Add a year and the version
Boon & Matthes, TADS-I (no year given)	Yes	—	Boon (2023), Norton; first pilot validation published 2025	n/a	Add a year; the validation evidence is preliminary
Hofmann, inverted protocol (cited through Leeds)	Yes	Book chapter	—	"Hofmann" hits in the database are Stefan G. Hofmann, a different author	Cite Hofmann (2009) in Luber (Ed.), pp. 313–328

Citation in the course	Exists	Peer-reviewed	Current?	Retracted?	Issue
"Old Day 1 slide notes after author review"; "the basic training"	Internal documents	No	—	—	Blocks 1.1 and 1.4 rely only on these. Acceptable for teaching method, but not evidence

Retraction search result: no DOI, author or title from the course's reference list appears in the Retraction Watch dataset. The one EMDR retraction found ("The Efficacy of EMDR in the Treatment of Depression", *J EMDR Practice & Research*) is not cited by the course.

6. Gaps: evidence and guidelines the course leaves out

- Current suicide-risk guidance.** NICE NG225 (2022) says: "Do not use risk assessment tools and scales to predict future suicide or repetition of self-harm", and do not use global low/medium/high stratification. It recommends needs-based assessment and risk formulation instead. See also Franklin et al. (2017), *Psychological Bulletin*, 143(2), 187–232.
- ISSTD guidelines.** "Guidelines for Treating Dissociative Identity Disorder in Adults, Third Revision" (2011), *Journal of Trauma & Dissociation*, 12(2), 115–187. This is the field's treatment guideline, it is still the current revision, and Leeds (p. 131) tells readers to review it. The course never cites it.
- The debate over phase-based treatment for CPTSD.** De Jongh et al. (2016), *Depression and Anxiety*, 33(5), 359–369, argue that the evidence for mandatory stabilization in CPTSD is methodologically limited. Cloitre (2016) replied in the same journal. The course teaches stabilization-first without mentioning this debate. For DID, phase-oriented treatment remains the ISSTD consensus.
- The progressive approach.** Gonzalez & Mosquera (2012) bring modified EMDR into the preparation phase. Leeds's glossary (p. 449) presents it as an alternative to the ISSTD model.
- Better screening tools.** DES-Taxon (Waller & Ross, 1997) and SDQ-20 or SDQ-5 (Nijenhuis et al., 1996, 1997) for somatoform dissociation, which the DES-II misses (Leeds pp. 130, 134).
- ICD-11 6B63, possession trance disorder.** This matters for Arab clients who describe experiences as jinn possession, and for cultural formulation in general.
- Arabic-validated instruments.** The course doesn't say whether the DES-II, MID or TADS-I have validated Arabic versions. I did not search for these; that needs a literature check.
- RDI cautions.** Patients with dismissing or fearful attachment may respond poorly (Leeds p. 161). Clinicians sometimes overuse RDI because of their own avoidance (Leeds p. 162), which fits the "therapist contribution" theme of block 2.5.
- Denial of diagnosis as a reason to postpone** reprocessing (Leeds p. 128).

7. Teaching quality

Structure: strong. - 20 blocks of 60 minutes each. Every block's timing table adds up to exactly 60 (checked). - The sequence runs: explanation, then skill, then worked example, then case task, then summary. - About 484 of 1,200 minutes (40%) are lecture; the rest is modelling, tasks and discussion. - Two continuing cases are revealed step by step (Reem R1-R12, Tarek T1-T12). The case record controls what is revealed when, and it is consistent with the decks. - Content warnings, check-in pauses, a designated assistant for participant distress, and minute-by-minute timings for each task are strengths.

Learning objectives: mostly good. - There are 42 objectives. Most use observable verbs: distinguish, classify, formulate, decide, choose, justify. - Four use verbs that can't be measured: - 2.2 "understand the function of avoidance" (تفهم) - 2.4 "become familiar with the language of collaboration" (تتعرف) - 2.5 "become familiar with your contribution" (تتعرف) - 4.4 "know the indications" (تعرف) - Rewrite these as observable behaviour, for example "name the probable function of an avoidance behaviour in a case

vignette". Continuing-education accreditors usually require measurable objectives; check the current EMDRIA credit criteria.

How well assessments match objectives: partial. - Most objectives have a matching written task (Tasks 1–21). - Three objectives have no task that assesses them: - 1.3 "explain action systems, ANP/EP and the three levels". Task 2 tests only generating a question. - 2.4 "explore the function of the part that wants to die". No task covers it. - 3.1 "build a small AIP map". It is assessed only by Task 11, which the manual marks optional and cut if time is short. - The 12-item pre-test/post-test cannot cover 42 objectives, and it was not supplied, so its alignment could not be assessed. - The course measures knowledge change, not changes in clinical behaviour.

Practical application: appropriate for the course's stated aim, and should be stated clearly. - All tasks are individual written case decisions. Demonstrations are read aloud by the trainer, and there is no bilateral stimulation and no practice in pairs. - This matches the course's stated aim of teaching decision-making, with specialized procedures covered only as indications and limits. - Certificates and marketing should say explicitly that the course does not confer competence in CIPOS, Loving Eyes, RDI, Flash or work with parts. The Day 4 closing slide already says these need separate training.

Language consistency. The trainer manual and workbook are in Modern Standard Arabic, while the slides and speaker notes are in Egyptian colloquial. This seems intentional, since the manual says there is a colloquial version of it. Confirm it suits a pan-Arab audience.

8. Where evidence is thin or conflicting

- **Predicting suicide.** The course follows Leeds (2016), but the later meta-analytic evidence and NICE guidance disagree with Leeds (see #19 and #22). This is a genuine conflict between sources, not a minor update.
- **RDI** has no high-quality controlled trial of the full protocol (Leeds p. 162). Hornsveld et al. (2011) called eye movements in RDI "questionable", and Leeds & Korn (2012) rejected that critique. The course should present it as contested.
- **Flash.** One RCT in a non-clinical population. The PTSD RCT (ENHANCE) had no published results in my search.
- **The DES-II** misses many dissociative disorders (Leeds p. 130).
- **The TADS-I** has only pilot validation (2025).
- **The structural dissociation model** is a theoretical model. Its levels are clinical constructs, not validated diagnostic categories. The trauma origin of dissociation is itself debated: Dalenberg et al. (2012) argue for it, and Lynn et al. (2014), *Psychological Bulletin*, 140(3), 896–910, argue it is overstated. Even the TTRD authors disagree about the ANP/EP terms (TTRD ch. 1). The course frames the model as "a tool for generating a question, not for classifying", which is the right stance. The spectrum slide (#7) is the one place it slips.
- **Phase-based treatment for CPTSD** is contested (De Jongh et al., 2016, versus Cloitre, 2016). For DID, phase-oriented treatment is consensus (ISSTD, 2011), but that consensus is expert-driven rather than based on RCTs.

9. Required edits, by priority

1. **Knife citation.** Cite one edition. The chapter numbers used fit the 2015 edition: CIPOS in ch. 13, Loving Eyes in ch. 11.
2. **IHD.** Replace the "Knife ch. 14" reference with the actual source.
3. **RDI (D3 S32).** Change "قليلة" (few) to "no high-quality controlled studies of the full protocol yet".
4. **Suicide risk (D1 S71, D2 S8, workbook).** Add: "No risk factor predicts suicide accurately; assess needs, not risk level" (NICE NG225, 2022; Franklin et al., 2017).
5. **Spectrum (D1 S29).** Label it as the structural dissociation model's view.

6. **Guidelines.** Add the ISSTD (2011) guidelines to the sources, and name the phase-based debate and the progressive approach.
7. **Screening (D1 S68-69).** Add DES-Taxon and SDQ-20/SDQ-5, and give years and versions for the SCID-D and TADS-I.
8. **ICD-11 6B63** (possession trance disorder): add it.
9. **Objectives.** Rewrite the four objectives that can't be measured, and add tasks, or remove the objectives, for 1.3 (explain), 2.4 (part that wants to die) and 3.1 (if Task 11 stays optional).
10. **Pre-test/post-test.** Supply it for an alignment review.